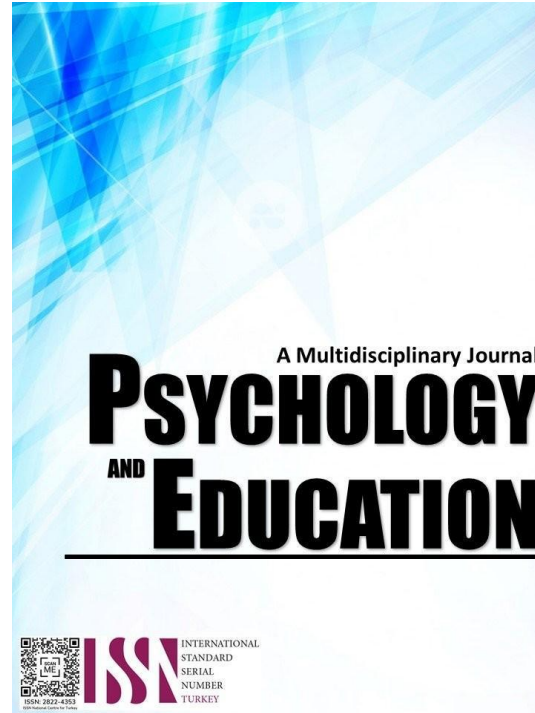


INTERSECTIONALITY OF NURSING CARE AND PSYCHOLOGICAL INTERVENTIONS



PSYCHOLOGY AND EDUCATION: A MULTIDISCIPLINARY JOURNAL

Volume: 57

Issue 8

Pages: 1055-1063

Document ID: 2026PEMJ5616

DOI: 10.70838/pemj.570810

Manuscript Accepted: 05-02-2026

Intersectionality of Nursing Care and Psychological Interventions

Armando Paolo Hofilena,* Calvin Ganub

For affiliations and correspondence, see the last page.

Abstract

This study aimed to examine how nurses in post-acute rehabilitation facilities integrated psychological interventions into the routine physical care nurses provide to their patients. A descriptive, phenomenological design was used to explore the experiences of eight Filipino and Filipino American nurses from three facilities. The instrument utilized for data collection involved a semi-structured interview guide that included open-ended questions designed to elicit responses regarding the conditions of the participants' patients, their mental health challenges, and the therapeutic strategies used to facilitate their care. Thematic analysis identified five themes describing the intersection of nursing care and psychological support: therapeutic communication as a foundation to care, prioritizing non-pharmacological interventions, behavioral and mood tracking as diagnostic tools, interdisciplinary collaboration, and adapting routines and maintaining interdependence of physical and psychological well-being. These findings indicated that nurses routinely incorporate psychological support in their routines through rapport-building communication with their patients, ongoing assessment of their physical and emotional states, and team-based care between nurses and other facility staff. However, limited formal training and inconsistent institutional support constrained the sustainability of these practices. Overall, the findings of the study highlight the need for targeted psychological skills training and stronger organizational support to enhance holistic and integrative care. These results also factored into the development of the Mindful Nursing Support Program, a training initiative designed to equip nurses with mental health assessment and intervention strategies for diverse patient populations.

Keywords: *nursing care, intersectionality, psychological interventions, post-acute rehabilitation, psychological support*

Introduction

Recent literature has demonstrated a unifying continuum between physical and mental health and well-being. Finch (2021) stated that these factors are interdependent, as supported by Hong et al. (2023), who found that lower levels of physical fitness correlate with poor psychological states. Physical activity has also been shown to mediate the relationship between physical and mental health, which implies that psychological interventions can indirectly enhance the physical recovery of their patients via influencing emotional resilience and motivation (White et al., 2024). Within this context, it is clear that nurses play an important role in integrating therapeutic communication, mood and behavior monitoring, and medication management into clinical practice, particularly in post-acute rehabilitation settings wherein patients face a vulnerable transition in their recovery. This is where psychological support would play a central role in nursing care within these facilities, which reflects the growing prominence of psychological interventions in primary and rehabilitative care (Williams et al., 2020). Furthermore, holistic approaches that address both physical and psychological needs have been proven to be effective in enhancing care quality and improving treatment outcomes (Balisi & Magallanes, 2023; Vasilyeva & Krupnov, 2020). Psychological interventions such as mood regulation, behavioral support, and psychotropic medication help in preventing the escalation of mental health concerns and support patients' physical health in return (Zacharia & Karekla, 2022; Brewer, 2023).

Although these factors highlight the importance of psychological support in nursing care, there remains a gap in understanding how nurses in post-acute rehabilitation facilities integrate said support into their routine practices. This is especially true when nurses do not have formal psychology degrees. Furthermore, there is little information on how cultural identity, clinical expertise, and consideration for patients' needs converge to shape the strategies used in nursing work. As such, this study is intended to investigate the lived experiences of Filipino and Filipino American nurses in post-acute rehabilitation facilities and examine how they employ therapeutic communication, psychotropic management, and mood and behavior monitoring within their rehabilitation care plans. Even without formal training, nurses play a role in supporting patients' mental health through practices embedded in their nursing duties. Insights from their experiences also revealed challenges and opportunities for improving therapeutic modalities and patient-centered care. These findings underscore the importance of integrating psychological interventions into nursing practice, informing policy, enhancing training programs, and strengthening psychosocial care within post-acute rehabilitation settings.

Research Questions

This study's primary purpose was to examine the intersectionality between nursing care and psychological interventions in post-acute rehabilitation settings. Specifically, this inquiry seeks to understand how Filipino and Filipino American nurses integrate psychological strategies into their daily clinical practice. Guided by the study's qualitative design, the central research question was as follows:

1. How do Filipino and Filipino American nurses in post-acute rehabilitation settings integrate psychological interventions in their daily nursing care?

Literature Review

Therapeutic Communication as Foundational to Care

Therapeutic communication has been consistently featured as a foundational element of nursing practice across various literature. This is because it strengthens trust, enhances engagement, and supports both physical and mental recovery. Abraham et al. (2024) emphasized its role in building collaborative relationships. In contrast, McAllister et al. (2021), Hartley et al. (2020), and Fellow (2021) collectively demonstrate that effective communication improves patient satisfaction, contributing to better physical and mental health outcomes. Furthermore, Curtiss et al. (2021) highlighted the importance of tailoring communication to patients' emotional states, which is an approach that aligns closely with the individualized, moment-to-moment adjustments that nurses make in post-acute rehabilitation settings. Ellison and Meyer (2020) further underscored therapeutic listening as a means of recognizing emotions and behavior, supporting the importance of interdisciplinary communication among mental health professionals. Taken together, these studies posit that therapeutic communication is essential as a tool that helps nurses in delivering holistic and psychologically informed care.

Prioritizing Non-Pharmacological Interventions

Non-pharmacological interventions have been identified by the literature as first-line strategies in mental health care, particularly when addressing anxiety, depression, and behavioral distress. Pinho et al. (2024) described these approaches as cost-effective and clinically essential, while McAllister et al. (2021) and Saijonkari et al. (2023) demonstrated their effectiveness in reducing maladaptive behaviors. Compared with general mental health settings, evidence from rehabilitation settings shows that short-term cognitive-behavioral interventions can produce meaningful improvements in emotional well-being for diverse populations (Zeng et al., 2024). Driscoll et al. (2021) also highlighted that these interventions may reduce reliance on high-dose psychotropic medications. Across these studies, non-pharmacological interventions—such as therapeutic communication, behavioral monitoring, relaxation strategies, and cognitive-behavioral techniques—are consistently framed as accessible, adaptable, and within nursing scope. This aligned directly with the ways nurses in post-acute rehabilitation integrate psychological support into daily care.

Behavior and Mood Tracking as Diagnostic Tools

Research consistently posited that nurses' continuous proximity to patients positions them as the first to detect subtle emotional or behavioral changes. This was supported by Forster et al. (2022), highlighting the diagnostic value of daily interactions, while Bai et al. (2021) and Lim et al. (2024) demonstrated that behavior and mood patterns can predict depressive episodes and next-day mood fluctuations. Yoshinaga et al. (2024) further emphasized that these observations guide individualized interventions. Standardized tools such as the Mental Health Observation Response Chart, PHQ-9, and GAD-7 (Maj et al., 2020) supported systematic assessment, while psychologists supplemented these with more specialized instruments. Across the reviewed literature, behavior and mood tracking is framed as both a diagnostic and therapeutic process—one that enables proactive, tailored care. This mirrors the role of Filipino and Filipino American nurses in this study, who rely on continuous observation to guide psychological interventions in rehabilitation settings.

Interdisciplinary Collaboration Enhances Psychological Support

Interdisciplinary collaboration is described by the literature as essential for addressing mental health needs in complex care environments. Din et al. (2024) described collaboration as a coordinated effort among nurses, psychologists, physicians, and social workers, while Baek et al. (2023) and McLaney (2022) highlighted that it compensates for the nurses' limited formal psychological training. Nurhidayah et al. (2022) linked collaborative practice to improved patient satisfaction, reduced complications, and better adherence to patients' treatment. Furthermore, Tyerman et al. (2021) and Feng and Yang (2022) demonstrated that collaboration mitigates systemic challenges such as stigma, workload pressures, and time constraints. These findings parallel the experiences of nurses in post-acute rehabilitation, who rely on interdisciplinary communication to integrate psychological interventions into routine care.

Adapting Routines and Maintaining Interdependence of Physical and Psychological Well-being

Across rehabilitation literature, physical and psychological well-being are described as interdependent components of recovery. Prusynski et al. (2024) emphasized that nurses are required to adapt routines to address both domains simultaneously, while Gutenbrunner et al. (2022) highlight the importance of long-term nurse–patient relationships in supporting emotional adjustment. Bondarchuk et al. (2023) showed that coping strategies taught by nurses promote problem-solving and reduce stress, and Prabakar (2024) reinforced that patients benefit when care addresses both physical and psychological needs. These findings aligned with the dual role of nurses in post-acute rehabilitation, who must adjust routines to support functional improvement and emotional stability.

Synthesis

Across the reviewed literature, five interconnected themes consistently emerged: therapeutic communication, non-pharmacological interventions, behavior and mood tracking, interdisciplinary collaboration, and the adaptation of routines to support both physical and psychological well-being. Collectively, these themes demonstrate that psychological support is an essential part of everyday nursing practice. The reviewed literature converges on the idea that nurses—particularly those in rehabilitation settings—use communication,

observation, and collaborative strategies to address emotional needs alongside physical recovery. As such, this synthesis directly reflects the findings of the present study, which show that Filipino and Filipino Nurses embed psychological interventions into routine care through intentional, culturally responsive, and clinically grounded practices.

Methodology

Research Design

This study utilized a descriptive, qualitative-phenomenological research design to examine how nurses in post-acute rehabilitation facilities would integrate psychological interventions into their daily practice. The use of a phenomenological approach was justified as it allowed for the research to explain and capture the essence of the participating nurses' lived experiences. This resulted in their perspectives guiding the inquiry rather than imposing external assumptions. Creswell and Creswell (2022) further justify the use of this approach as phenomenology provides a systematic means of exploring how individuals would interpret the meaning of a phenomenon within the context of their professional roles. Furthermore, the phenomenological approach proved appropriate in the research study's aim of understanding the interplay between psychosocial care in post-acute rehabilitation nursing. By focusing on the nurses' firsthand accounts, the research was able to illuminate how psychological considerations are embedded in routine clinical decision-making, the challenges encountered in this integration, and the strategies nurses utilize in addressing them.

Participants

The participants of the study included eight Filipino and Filipino American registered nurses employed at three post-acute rehabilitation facilities in Illinois. Convenience sampling was used to select participants whose dual role addressed both physical and psychological needs of recovering from surgery, stroke, or chronic conditions. The sample was gender-balanced (four male, four female), with ages ranging from 29 to 56 years and clinical experience spanning 5 to 35 years. The exclusion criteria of the study ruled out nurses of Western or European descent, nurses practicing outside Illinois, nurses who were 67 and above, and nurses working in hospital or school-based settings. Although the attained sample reflected the study's population of interest, the use of convenience sampling and focus on Filipino and Filipino American nurses in a single stay may limit broader representation on the topic. Therefore, the findings presented reflect the experiences of this particular group and may not fully capture the perspectives of nurses from other cultural backgrounds, geographical locations, or practice settings.

Instrument

The instrument used for the study's data collection was a semi-structured interview guide made to explore nurses' perspectives on patient admissions to post-acute rehabilitation facilities and the integration of psychological interventions into their care. The guide consisted of open-ended questions that were designed to elicit detailed responses pertaining to the conditions of patients, their mental health challenges, and the therapeutic strategies used to facilitate their care. The study was guided by this overarching research question: What are the reasons why patients are admitted to post-acute rehabilitation facilities?"

Probing questions were included to deepen the discussion and obtain richer descriptions of the nurses' experiences. Sample probing questions included: "Are there any mental conditions that you have noticed from time to time with the patients?" and "What kind of patients do you deal with?" These questions encouraged participants to reflect on their communication practices, the types of patients they cared for, and the psychological needs encountered in transitional care settings. Additional items addressed therapeutic communication and prompted nurses to describe how they supported patients' mental health during recovery. The flexible structure of the interview ensured consistency across interviews while allowing individual experiences and insights to emerge. Demographic factors such as age, sex, and years of experience were also included due to their potential to reflect on how nurses approached patient care.

Procedure

The data gathering procedure began with securing approval from the administrators of the three post-acute rehabilitation facilities where the study was conducted. The approval was requested through direct contact with the administrators. After approval was obtained, the administrators assisted in identifying registered nurses who met the inclusion criteria for the participants. These nurses were then invited to participate in the study, with informed consent being obtained during individual orientation sessions. Convenience sampling was used to recruit participants, and interviews were conducted and continued until saturation was reached. Each interview followed the semi-structured instrument that guided the study and was conducted in a private setting within the facilities. After each interview, the participants were engaged in a brief debriefing to clarify their responses and provide feedback on the interview process.

All interviews were then transcribed verbatim and analyzed inductively. Coding procedures incorporated bracketing to minimize researcher assumptions, and external audits were conducted to support confirmability. Credibility of the data was strengthened through triangulation, member checking, and peer debriefing. The researcher also maintained reflectivity via a journal of assumptions, insights, and reflections, which was used throughout the study. Furthermore, transferability and dependability were enhanced through detailed descriptions of the research process and the maintenance of a clear audit trail. All collected data was then securely stored and accessible only to authorized reviewers, in full compliance with ethical standards of confidentiality.

Data Analysis

Data analysis employed thematic procedures outlined by Cresswell and Cresswell (2022). The process began with organizing and preparing the interview transcripts, which were transcribed verbatim and reviewed multiple times to develop a holistic understanding of the data. After the initial reviews, significant statements were coded, and the aforementioned codes were grouped into patterns reflecting recurring ideas across participants. These patterns were then developed into themes that represented the study's core findings. Interrater and intercoder reliability procedures were also utilized to support the verifiability of the coding process. The constructivist paradigm guiding the study also informed the analysis by emphasizing the participants' ontologies, epistemologies, and axiologies. This allowed the researcher to examine how nurses' beliefs, knowledge, and values shaped their descriptions of psychological interventions and nursing care in post-acute rehabilitation settings.

Trustworthiness was established through credibility, dependability, and confirmability. Credibility was attained through member checking, where participants reviewed the accuracy of the study's interpretations. Dependability was strengthened through maintaining detailed field notes, complete records, and consistent documentation throughout the study. Confirmability was enhanced through an audit trail and the researcher's reflexive journaling, which documented assumptions, insights, and decisions during analysis. All data were securely stored and accessible only to authorized reviewers in accordance with confidentiality standards.

Ethical Considerations

To ensure that ethical considerations were given top priority, this research underwent ethics review and approval from the Research Ethics Review Committee (RERC). All participants provided informed consent and were assured of voluntary participation, the right to withdraw, and minimal study-related risks. Privacy and confidentiality were also maintained in accordance with the Data Privacy Act of 2012, with all the identifying information securely stored and scheduled for disposal in three years after the study's conclusion.

Potential vulnerabilities were also mitigated through flexible scheduling, confidentiality assurances, and access to mental health support if needed, though no such incidents have occurred. Ethical principles of beneficence, justice, and respect for persons were upheld throughout the research process. To ensure that the study maintained its objectivity, the study employed reflexive bracketing and an audit trail as a means of minimizing personal bias and to have the participants' perspectives represented accurately.

Results and Discussion

This section presents the findings of the study, beginning with short case vignettes of the participants to provide context, followed by the thematic analysis of their lived experiences. Each of the themes has a table wherein Significant statements were identified, interpreted for meaning, and grouped into themes and topics, as summarized in Tables 2 to 6.

Case Vignettes

Fu is a 55-year-old male nurse with over 25 years of experience. He is a former director of nursing who returned to floor work. In his interview, he emphasized reassurance, encouragement, and participation in social and rehabilitation activities to support patients' emotional stability.

Alya is a 37-year-old female nurse with 5 years of experience. She works primarily with patients with dementia and mobility issues. In her interview, she highlighted consistent reorientation and clear reminders to reduce confusion and support daily functioning.

Adrien is a 29-year-old male with 7 years of experience, having specialized in orthopedic rehabilitation. In his interview, he stated that he had focused on structured routines and patient motivation during recovery.

Rose is a 55-year-old female with 16 years of experience, having worked in home health and dementia care. In her interview, she explained having used familiarity, patience, and routine to support cognitively impaired patients.

Gabriel is a 45-year-old male with 21 years of experience, involving administrative and bedside work. He is currently an admissions director. In his interview, he explained having integrated psychosocial awareness into admission assessments and ongoing care coordination.

Nathalie is a 54-year-old female with 8 years of experience, having had a background in transitional care. In her interview, she stated having prioritized psychosocial and physical assessments, family involvement, and active listening to support patient adjustment.

Kim is a 54-year-old male with 34 years of experience, having been a long-term rehabilitation nurse who worked with orthopedic and cardiac patients. In his interview, he stated he valued steady bedside presence, clear explanations, and continuous behavior and mood monitoring when treating patients.

Juleka is a 56-year-old female with 15 years of experience, having a psychiatric and rehabilitation background, including memory care. In her interview, she explained having applied psychiatric nursing skills to support elderly patients with dementia and complex behavioral needs.

Table 1. *Profile of the Participants*

<i>Participant</i>	<i>Age</i>	<i>Length of Work Experience</i>	<i>Sex</i>
Fu	55	25	Male
Alya	37	5	Female
Adrien	29	7	Male
Rose	56	16	Female
Gabriel	45	21	Male
Nathalie	54	8	Female
Kim	54	35	Male
Juleka	56	15	Female

The results in Table 1 represent the range of ages and experience levels of the participants. The even sex distribution and variety of post-acute rehabilitation facility settings provide a diverse perspective on nursing care practices in post-acute rehabilitation. Building on the vignettes, the following discussion presents the themes that emerged across participants' narratives. These themes highlight recurring strategies, challenges, and values that shaped how nurses delivered psychological support alongside medical care.

Theme 1: Therapeutic Communication as Foundational to Care

This theme highlights therapeutic communication as a major foundation of holistic nursing care in post-acute rehabilitation facilities. Participants consistently described therapeutic communication as the basis of effective rehabilitation nursing. This is because it served as a therapeutic process that built trust, reduced anxiety, and encouraged engagement. As such, nurses emphasized active listening, empathy, humor, and consistent presence as strategies that helped patients feel safe and emotionally grounded. These findings also aligned with the reviewed literature, stating that therapeutic communication enhances patient engagement and satisfaction (McAllister et al., 2021; Hartley et al., 2020) and deepens nurses' understanding of patients' emotional states (Curtiss et al., 2021; Ellison & Meyer, 2020).

Table 2. *Participants, Significant Statement, Meaning from Statement: Therapeutic Communication as Foundation to Care*

<i>Participants</i>	<i>Significant Statement</i>	<i>Meaning from Statement</i>
Nathalie	"Active listening is usually the best thing that helps the patients with mental health problems."	Listening builds trust and reassures patients, making them feel safe.
Adrien	"Active listening... giving encouragement, empathy, humor... helps lighten the mood."	Emotional engagement helps improve the mood of patients.
Juleka	"Therapeutic communication...helps a lot because they know how to...make them motivated more... which is better for them."	Therapeutic communication fosters motivation and patient cooperation.
Alya	"Using therapeutic communication helps you understand how to approach them... you get to know your patient more."	Communication allows understanding of patient behavior and routine.
Fu	"A lot of these patients just need someone to talk to... they get lonely."	Providing presence and empathy helps relieve emotional distress.
Gabriel	"It kind of helps us get their trust... they feel safe with somebody they can trust."	Building rapport through communication enhances safety and comfort.

Theme 2: Prioritizing Non-Pharmacological Interventions

This theme reflects how nurses in post-acute rehabilitation facilities prioritize non-pharmacological methods before resorting to medication. These interventions—which included structured routines, music, social engagement, and therapeutic dialogue—are prioritized as the first line of support for the patients before considering psychotropic medication (Pinho et al., 2024). This is reflected by the fact that such interventions are effective in reducing anxiety, depression, and behavioral distress (Saijonkari et al., 2023). The participants of the study also supported this notion, their interviews emphasizing that prioritizing such interventions can help in facilitating engagement towards the treatment while minimizing risks associated with psychotropic use (McAllister et al., 2021).

Table 3. *Participants, Significant Statement, Meaning from Statement: Prioritizing Non-Pharmacological Interventions First*

<i>Participants</i>	<i>Significant Statement</i>	<i>Meaning from Statement</i>
Alya	"We try to use activities, companionship, and music first before giving medications."	Nurses prioritize holistic, non-drug approaches to calm and engage patients.
Fu	"We try to be more conservative first... communication is very important before we go to medication."	Non-drug methods such as conversation and encouragement are primary approaches.
Adrien	"Simply having routines, like opening their blinds, bringing them to the dining room to eat, or playing music to them, makes a big difference."	Structured routines and sensory stimulation reduce anxiety and promote engagement.
Kim	"We involve psychologists and social workers... sometimes all they need is someone to listen."	Psychological and social approaches are prioritized before pharmacologic ones.
Rose	"We manage the psychotropic meds... but we prevent patients from needing to go to the hospital."	Non-pharmacological care helps reduce escalation and hospitalization.
Juleka	"We encourage them to join activities so they won't feel isolated"	Companionship and participation reduce loneliness and agitation.

Theme 3: Behavior and Mood Tracking as Diagnostic Tools

This theme highlights behavior and mood tracking as an important diagnostic tool in post-acute rehabilitation facilities. Participants described behavior and mood tracking as essential for detecting early signs of distress, monitoring progress, and adjusting interventions. Nurses compare the current behaviors of their patients to baseline assessments before communicating changes to families and care teams, and use observations to guide treatment decisions. These findings aligned with literature showing that behavioral patterns predict depressive episodes and mood fluctuations (Bai et al., 2021; Lim et al., 2024) and that standardized tools support systematic assessment (Maj et al., 2020). Behavior and mood tracking can also help facilitate specific interventions to suit the patients' needs, especially when supported by standardized tools (Yoshinaga et al., 2024; Villarreal-Zegarra et al., 2023). This form of tracking thus functions as both a diagnostic and therapeutic tool in rehabilitation.

Table 4. *Participants, Significant Statement, Meaning from Statement: Behavior and Mood Tracking as Diagnostic Tool*

Participants	Significant Statement	Meaning from Statement
Kim	"It's pretty much a lot of nursing observations and documentation... that's pretty much one of the keys that help treat patients for both."	Continuous observation and documentation guide interventions and treatment.
Rose	"We check for changes in mood from one shift to another to update the care plan."	Tracking behavior and emotions ensures quick response to patient needs.
Alya	"We have a baseline care plan upon admission...we study their behavior if we see any changes."	Establishing behavioral baselines help detect deviations early.
Adrien	"We monitor changes and make sure their depression isn't getting worse."	Ongoing mood checks allow timely adjustments and prevent relapse.
Fu	"If there's any changes in mood and behavior then of course we let the family know first and then we try to assess if these behaviors were present before or if these are new."	Prioritizing communication with family or primary caregivers first.
Gabriel	"We monitor them just to see if there are any changes. If the condition is getting worse or stable."	Consistent mood observation identifies progress or deterioration.

Theme 4: Interdisciplinary Collaboration Enhances Psychological Support

This theme highlights interdisciplinary collaboration as an important mechanism for strengthening psychological support for patients. Participants emphasized that psychological support is strengthened through collaboration with other healthcare staff in the setting, being described as important in preventing regression within patients and helping them recognize when they have to adjust medications and alter interventions to ensure proper care for patients. Nurses' bedside observations were also used to inform medication adjustments, therapy modifications, and coordinated care planning, which also helps healthcare staff such as psychiatrists, psychologists, physicians, and therapists. The literature also reflected the benefits of interdisciplinary collaboration as it compensates for limited psychological training (Baek et al., 2023), improves adherence and outcomes (Nurhidayah et al., 2022), and reduces systemic barriers such as stigma and workload (Tyerman et al., 2021; Feng & Yang, 2023).

Table 5. *Participants, Significant Statement, Meaning from Statement: Interdisciplinary Collaboration Enhances Psychological Support*

Participants	Significant Statement	Meaning from Statement
Fu	"Collaboration with the psychologist and social worker is important so patients don't regress."	Nurses recognize team coordination as key to sustained recovery.
Adrien	"We report our observations to the doctor so medications and therapies can be adjusted."	Nursing input informs medical and psychological care decisions
Rose	"We collaborate with the psychiatrist if there needs to be adjustments."	Communication between professionals ensures appropriate medication management.
Alya	"Communication is the key...you really have to communicate with the doctor, psychiatrist, and others."	Interprofessional communication improves alignment of care.
Kim	"It's hard, but if you involve the whole care team it's a big help... it supports both nurses and patients."	Shared responsibility among the team enhances support and outcomes
Gabriel	"We have in-house psychologists and psychiatrists that we update when it comes to patients having elevated mood problems."	Collaboration allows timely interventions for psychological issues.

Theme 5: Adapting Routines and Maintaining Interdependence of Physical and Psychological Well-being

This theme focuses on adapting routines and maintaining the interdependence of physical and psychological. Participants described these factors as important, emphasizing that physical progress is inseparable from a person's psychological state, noting that mental health challenges like anxiety, low motivation, and emotional distress can prevent patients from meaningfully engaging in therapy. The participants also reported that adjusting treatment pacing, communication styles, and daily routines to address both physical and psychological well-being helped ensure that the patients' care remained holistic and responsive. These statements align with related literature showing that psychological stability enhances participation in physical rehabilitation (Chou & Tseng, 2020; White et al., 2024) and that holistic approaches improve overall recovery (Tusaie & Fitzpatrick, 2022).

Table 6. *Participants, Significant Statement, Meaning from Statement: Adapting Routines and Maintaining Interdependence of Physical and Psychological Well-being*

Participants	Significant Statement	Meaning from Statement
Adrien	"Different compared to long-term because you have a short time to get to know them... you don't really get as much time to learn them."	Short-term settings require flexibility and quick understanding of patients.
Nathalie	"If they are anxious, they cannot go to therapy. That makes sense, and so we will ask: what is going on, man?"	The psychological need of the patient can influence or affect the physical needs of patients.
Kim	"We have to adjust physical treatment with the mental condition. You cannot just treat one and disregard the other."	Integration of physical and mental care ensures holistic healing.
Alya	"Balancing physical and psychological aspects when dealing with patients is hard."	Nurses struggle to provide equal attention to both areas within limited time.
Rose	"We see them as a whole... therapists and nurses make sure everything is covered. It's teamwork."	Holistic approach promotes comprehensive recovery.
Fu	"It's challenging but involving the whole team helps...dealing with both physical and medications alone is a lot."	Team involvement lightens workload and supports patient well-being.

Thematic Insight

Across all themes, the findings show that psychological support is embedded in routine nursing practice rather than functioning as a separate domain. Nurses utilized therapeutic communication to build trust, non-pharmacological interventions to stabilize mood, and behavior tracking to guide individualized care. Interdisciplinary collaboration ensured coordinated treatment, while adaptive routines maintained the interdependence of physical and psychological well-being. Overall, these integrated practices reflect a holistic, culturally responsive model of rehabilitation nursing consistent with current literature. Figure 1 summarizes how these themes function as core components of nursing care, affirming that patient recovery is achieved through practices and interventions that support physical and psychological conditions.

Figure 1. *Simulacrum*

Conclusions

This study demonstrates that psychological interventions are an integral factor for routine nursing practice in post-acute rehabilitation settings. The participating Filipino and Filipino American nurses described having consistently incorporated therapeutic communication, non-pharmacological strategies, and tracking and monitoring of mood and behavior into daily care. This demonstrated a holistic approach that attends to both physical recovery and psychological health. Interdisciplinary collaboration furthers this integration, emphasizing the value of coordinated, culturally responsive care in rehabilitative facilities. These findings also affirm that psychological interventions are essential to post-acute nursing practice, enacted through intentional and multifaceted strategies that promote comprehensive patient care. The results of the study have informed and recommended the development of the Mindful Nursing Support Program, a proposed training initiative aimed at strengthening nurses' competence in delivering evidence-based psychological interventions as part of standard rehabilitation care. Further research is also recommended to expand the topics examined by this study with other nursing populations, and to explore how interdisciplinary education can effectively embed competencies such as therapeutic listening and behavior tracking into foundational nursing training.

References

- Abraham, S., Nsatimba, F., Agyare, D., Agyeiwaa, J., Opoku-Danso R., Ninnoni, J., Doe, P., Kuffour, B., Anumel, B., Berchie, G., Boso, C., Druye, A., Okantey, C., Owusu, G., Obeng, P., Amoadu, M., Commey, I. (2024). Barriers and outcomes of therapeutic communication between nurses and patients in Africa: A scoping review. <https://doi.org/10.1186/s12912-024-02038-0>
- Baek, H., Han, K., Cho, H., & Ju, J. (2023). Nursing teamwork is essential in promoting patient-centered care: A cross-sectional study. *BioMed Central Nursing*, 22(1), 433. <https://doi.org/10.1186/s12912-023-01592-3>
- Bai, R., Xiao, L., Guo, Y., Zhu, X., Li, N., Wang, Y., Chen, Q., Feng, L., Wang, Y., Yu, X., Wang, C., Hu, Y., Liu, Z., Xie, H., & Wang, G. (2021). Tracking and monitoring mood stability of patients with major depressive disorder by machine learning models using passive digital data: Prospective naturalistic multicenter study. *JMIR MHealth and Uhealth*, 9(3), e24365. <https://doi.org/10.2196/24365>
- Balisi, A. N., & Magallanes, C. I. (2023). Relationship between pain catastrophizing and perceived wellness among chronic pain patients. *Philippine Social Science Journal*, 6(1), 9-17. <https://doi.org/10.52006/main.v6i1.703>
- Bondarchuk, O., Balakhtar, V., Pinchuk, N., Pustovalov, I., Pavlenok, K. (2023). Adaptation of coping strategies to reduce the impact of stress and loneliness on the psychological well-being of adults. <https://lib.iitta.gov.ua/id/eprint/738204/>
- Brewer, A. (2023). Painful feelings: Opioids as tools for avoiding emotional labor in Hospital Work. *Journal of Health and Social Behavior*, 64(3), 386–400. <https://doi.org/10.1177/00221465231176077>
- Chou, H. J., & Tseng, K. Y. (2020). The experience of emergency nurses caring for patients with mental illness: A qualitative study. *International Journal of Environmental Research and Public Health*, 17(22), 8540.
- Creswell, J. W., & Creswell, J. D. (2022). *Research design: Qualitative, quantitative, and mixed methods approaches* (6th ed.). SAGE Publications.
- Curtiss, J. E., Levine, D. S., Ander, I., & Baker, A. W. (2021). Cognitive-behavioral treatments for anxiety and stress-related disorders. *Focus (American Psychiatric Publishing)*, 19(2), 184–189. <https://doi.org/10.1176/appi.focus.20200045>
- Din, B. Anjum, F., Malik, A. (2024). Cross-disciplinary collaboration in healthcare: Enhancing outcomes through team-based care. <https://www.researchcorridor.org/index.php/mjh/article/view/43>
- Driscoll, M. A., Edwards, R. R., Becker, W. C., Kaptchuk, T. J., & Kerns, R. D. (2021). Psychological interventions for the treatment of chronic pain in adults. *Psychological Science in the Public Interest: A Journal of the American Psychological Society*, 22(2), 52–95. <https://doi.org/10.1177/15291006211008157>
- Ellison, D. L., & Meyer, C. K. (2020). Presence and therapeutic listening. *The nursing clinics of North America*, 55(4), 457–465. <https://doi.org/10.1016/j.cnur.2020.06.012>
- Fellow, C. (2021). Therapeutic communication within the nurse–patient relationship: A concept analysis. <https://doi.org/10.1111/ijn.12938>
- Feng, L., & Yang, D. (2022). Observation on the effect of high-quality nursing intervention plus health education in chemotherapy for non-small cell lung cancer and its influence on the physical and mental health of patients. *Evidence-based complementary and alternative medicine*.
- Finch, J. (2021). Mental health and wellbeing exist along a continuum. Centre for Clinical Psychology Melbourne. <https://ccp.net.au/mental-health-and-wellbeing-exist-along-a-continuum/>
- Forster, J. A., Coventry, A. A., & Daniel, C. (2022). Optimizing the response to mental health deterioration: Nurses experiences of using a mental health observation response chart. *International Journal of Mental Health Nursing*, 32(1). <https://doi.org/10.1111/inm.13062>
- Gutenbrunner, C., Stievano A., Nugraha B., Stewart D. & Catton H. (2022). Nursing – a core element of rehabilitation. *International Nursing Review*. 69, 13–19
- Hartley, S., Raphael, J., Lovell, K., & Berry, K. (2020). Effective nurse-patient relationships in mental health care: A systematic review of interventions to improve the therapeutic alliance. *International Journal of Nursing Studies*, 102(1), 1–9. <https://doi.org/10.1016/j.ijnurstu.2019.103490>
- Hong, Y., Shen, J., Hu, Y., Gu, Y., Bai, Z., Chen, Y., & Huang, S. (2024). The association between physical fitness and mental health among college students: A cross-sectional study. *Frontiers in Public Health*, 12. <https://doi.org/10.3389/fpubh.2024.1384035>
- Lim, D., Jeong, J., Song, Y. M., Cho, C.-H., Yeom, J. W., Lee, T., Lee, J.-B., Lee, H.-J., & Kim, J. K. (2024). Accurately predicting mood episodes in mood disorder patients using wearable sleep and circadian rhythm features. *NPJ Digital Medicine*, 7(1). <https://doi.org/10.1038/s41746-024-01333-z>

- Maj, M., Stein, D. J., Parker, G., Zimmerman, M., Fava, G. A., De Hert, M., Demyttenaere, K., McIntyre, R. S., Widiger, T., & Wittchen, H. U. (2020). The clinical characterization of the adult patient with depression aimed at personalization of management. *World Psychiatry: Official Journal of the World Psychiatric Association (WPA)*, 19(3), 269–293. <https://doi.org/10.1002/wps.20771>
- McAllister, S., Simpson, A., Tsianakas, V., Canham, N., De Meo, V., Stone, C., & Robert, G. (2021). Developing a theory-informed complex intervention to improve nurse–patient therapeutic engagement employing experience-based co-design and the behaviour change wheel: an acute mental health ward case study. *BMJ Open*, 11(5), e047114.
- McLaney, E., Morassaei, S., Hughes, L., Davies, R., Campbell, M., & Di Prospero, L. (2022). A framework for interprofessional team collaboration in a hospital setting: Advancing team competencies and behaviours. *Healthcare Management Forum*, 35(2), 112–117.
- Nurhidayah, R. E., Tanjung, H. R., & Martina, S. J. (2022). Team effectiveness in interprofessional collaboration. Novateur Publication.
- Pinho, L., Fonseca, C., Gawęda, L., Lopes, M., Schneider, B. (2024). Editorial: Non-pharmacological interventions for mental disorders. <https://doi.org/10.3389/fpsyg.2024.1363348>
- Prabakar, A. D. (2024). The Power of Thought: The Role of Psychological Attentiveness and Emotional Support in Patient Trajectories. *The Yale journal of biology and medicine*, 97(3), 335–347. <https://doi.org/10.59249/CPTG1770>
- Prusynski, R. A., Rundell, S. D., Pradhan, S., & Mroz, T. M. (2022). Some but not too much: multiparticipant therapy and positive patient outcomes in skilled nursing facilities. *Journal of Geriatric Physical Therapy*, Publish Ahead of Print. <https://doi.org/10.1519/jpt.0000000000000363>
- Saijonkari, M., Paronen, E., Lakka, T., Tolmunen, T., Linnosmaa, I., Lammintakanen, J., Isotalo, J., Rekola, H., Mäki-Opas, T. (2023). Promotive and preventive interventions for mental health and well-being in adult populations: A systematic umbrella review. <https://doi.org/10.3389/fpubh.2023.1201552>
- Tusaie, K., & Fitzpatrick, J. J. (2022). Advanced practice psychiatric nursing: Integrating psychotherapy, psychopharmacology, and complementary and alternative approaches across the life span. Springer Publishing Company.
- Tyerman, J., Patovirta, A. L., & Celestini, A. (2021). How stigma and discrimination influences nursing care of persons diagnosed with mental illness: A systematic review. *Issues in Mental Health Nursing*, 42(2), 153–163. <https://doi.org/10.1080/01612840.2020.1789788>
- Vasilyeva, E., & Krupnov. (2020). Development of the methodological approach to the comprehensive assessment of the innovative project effectiveness. <https://doi.org/10.1051/e3sconf/202016410037>
- Villarreal-Zegarra, D., Barrera-Begazo, J., Otazú-Alfaro, S., Mayo-Puchoc, N., Bazo-Alvarez, J. C., & Huarcaya-Victoria, J. (2023). Sensitivity and specificity of the Patient Health Questionnaire (PHQ-9, PHQ-8, PHQ-2) and General Anxiety Disorder scale (GAD-7, GAD-2) for depression and anxiety diagnosis: a cross-sectional study in a Peruvian hospital population. *BMJ Open*, 13(9), e076193. <https://doi.org/10.1136/bmjopen-2023-076193>
- White, R.L., Vella, S., Biddle, S., Sutcliffe, J., Guagliano, J., Uddin, R., Burgin, A., Apostopoulos, M., Nguyen, T., Young, C., Taylor, N. Lilley, S. & Teychenne M. (2024). The relationship between physical and mental health: A mediation analysis. *Social Science & Medicine*, 195(0277-9536), 42–49. <https://doi.org/10.1016/j.socscimed.2017.11.008>
- Williams, R., Jenkins, D. A., Ashcroft, D. M., Brown, B., Campbell, S., Carr, M. J., Cheraghi-Sohi, S., Kapur, N., Thomas, O., Webb, R. T., & Peek, N. (2020). Diagnosis of physical and mental health conditions in primary care during the COVID-19 pandemic: A retrospective cohort study. *The Lancet. Public health*, 5(10), e543–e550. [https://doi.org/10.1016/S2468-2667\(20\)30201-2](https://doi.org/10.1016/S2468-2667(20)30201-2)
- Yoshinaga, N., Obara, Y., Kawano, N., Kondo, K., Hayashi, Y., Nakai, M., Takeda, R., & Tanoue, H. (2024). Real-World effectiveness and predictors of nurse-led individual cognitive behavioral therapy for mental disorders: An updated pragmatic retrospective cohort study. *Behavioral Sciences*, 14(7), 604–604. <https://doi.org/10.3390/bs14070604>
- Zacharia, M., & Karekla, M. (2022). The role of psychologists and psychological approaches in cancer care. In *Handbook of quality of life in cancer*. Springer International Publishing.
- Zeng, G., Niu, J., Zhu, K., Li, F., Li, L., Gao, K., Zhuang, Y., Zhang, B., Han, X., Ye, G., Gao, Z., & Li, H. (2024). Effects of non-pharmacological interventions on depressive and anxiety symptoms in pregnant women: a systematic review and network meta-analysis. *EClinicalMedicine*, 79, 103011. <https://doi.org/10.1016/j.eclinm.2024.103011>

Affiliations and Corresponding Information

Armando Paolo Hofilena

University of St. La Salle – Philippines
aphofilena0709@gmail.com

Calvin Ganub

University of St. La Salle – Philippines